

King Fahad Central Hospital

Acute Ischemic Stroke Admission Order Set

Aligned with AHA/ASA Get With The Guidelines–Stroke and Joint Commission Primary/Comprehensive Stroke Center performance measures

Patient Name: _____	Date/Time of Admission: _____
DOB: _____ MRN: _____	Last Known Well (LKW): _____
Admitting Physician: _____	Weight (kg, measured): _____
Allergies: _____	Code Status: Full Code / DNR / DNI / Other: _____

Admitting Diagnosis: ☐ Acute Ischemic Stroke ☐ TIA ☐ Intracerebral Hemorrhage ☐ Stroke, etiology TBD

1. Core Performance Measure Checklist (Get With The Guidelines–Stroke)

To be verified by stroke coordinator/nursing prior to discharge; drives Joint Commission/AHA certification reporting.

- ☐ Dysphagia screen completed before any oral intake (food, fluids, or medication)
- ☐ NIH Stroke Scale (NIHSS) documented on arrival and per protocol
- ☐ Venous thromboembolism (VTE) prophylaxis ordered by end of hospital day 2
- ☐ Antithrombotic therapy given by end of hospital day 2
- ☐ Anticoagulation ordered for documented atrial fibrillation/flutter (if not contraindicated)
- ☐ Thrombolytic (IV alteplase/tenecteplase) administered if eligible and within window
- ☐ High-intensity statin therapy ordered (LDL $\geq$ 70 or per guideline)
- ☐ Stroke education provided to patient/caregiver (risk factors, warning signs, follow-up)
- ☐ Assessed for rehabilitation services (PT/OT/Speech)
- ☐ Smoking cessation counseling/therapy offered if applicable
- ☐ Plan for follow-up after discharge documented

2. Vital Signs & Neurological Monitoring

- ☐ **Neuro checks + vital signs: q15min x 2h, then q30min x 6h, then q1h x 16h (post-tPA protocol) — adjust per unit protocol**
- ☐ Neuro checks + vital signs q1–2h if not thrombolysed (per unit/ICU protocol)
- ☐ Continuous cardiac (telemetry) monitoring x 24–48h minimum to detect atrial fibrillation
- ☐ Continuous pulse oximetry; supplemental O2 to keep SpO2 > 94%
- ☐ Strict intake and output
- ☐ Notify physician for SBP >185 mmHg or DBP >110 mmHg before IV thrombolysis (tPA), or SBP >180 mmHg or DBP >105 mmHg during the first 24 hours after IV thrombolysis, per stroke protocol., temp >38.0°C, HR <50 or >120, SpO2 <94%, new/worsening neurologic deficit, decreased LOC, severe headache, NIHSS increase $\geq$ 2 points

3. Activity & Positioning

- ☐ Bed rest / HOB per protocol — flat or as tolerated x 24h if hypoperfusion physiology suspected (physician to specify)
- ☐ HOB elevated $\geq$ 30° once stable, to reduce aspiration risk
- ☐ Fall precautions
- ☐ Aspiration precautions
- ☐ Bedside swallow screen prior to first oral intake; formal Speech-Language Pathology evaluation if screen fails or NIHSS/exam concerning
- ☐ PT/OT evaluation within 24–48h of admission
- ☐ Sequential compression devices (SCDs) for VTE prophylaxis while immobile
- ☐ Out of bed to chair as tolerated once medically stable — specify: _____

4. Diet

- ☐ **NPO until formal dysphagia screen completed**
- ☐ If screen passed: Diet — ☐ Regular ☐ Cardiac/low-sodium ☐ Diabetic ☐ Other: _____
- ☐ If screen failed: NPO, Speech-Language Pathology consult, consider IV fluids/alternate nutrition route
- ☐ Aspiration precautions if dysphagia confirmed

5. IV Fluids

- ☐ IV access: 1–2 peripheral IVs
- ☐ Isotonic IV fluid (e.g., 0.9% NaCl) at _____ mL/hr — avoid hypotonic and dextrose-containing fluids acutely unless hypoglycemic
- ☐ Avoid hypotonic fluids (may worsen cerebral edema)

6. Laboratory Studies

- ☐ CBC with differential
- ☐ Basic or comprehensive metabolic panel
- ☐ PT/INR, aPTT
- ☐ Fasting lipid panel
- ☐ Hemoglobin A1c
- ☐ Troponin
- ☐ Type and screen (if hemorrhagic conversion risk or thrombectomy candidate)
- ☐ Urinalysis / urine or serum pregnancy test if applicable
- ☐ Additional (hypercoagulable panel, toxicology, etc.) if clinically indicated: _____

7. Imaging & Cardiac Workup

- ☐ non-contrast head CT — completed pre-admission per acute stroke protocol
- ☐ CT angiography head/neck or MR angiography (large vessel occlusion assessment)
- ☐ MRI brain with diffusion-weighted imaging
- ☐ Carotid duplex ultrasound (if not already characterized by CTA/MRA)
- ☐ 12-lead ECG
- ☐ Transthoracic echocardiogram ($\pm$ bubble study/TEE if cryptogenic or embolic source suspected)
- ☐ Continuous/extended cardiac rhythm monitoring after discharge if etiology undetermined (event monitor)

8. Medications

8a. Thrombolysis / reperfusion (if applicable)

- ☐ IV alteplase/tenecteplase per institutional acute stroke protocol — dose/time per protocol, weight-based
 - ☐ Post-thrombolysis: hold antiplatelet/anticoagulant agents for 24h and repeat head CT before starting, per protocol
- Endovascular thrombectomy per Neurointerventional/Comprehensive Stroke Center protocol if large-vessel occlusion and eligible.*

8b. Antithrombotic therapy (start within 24–48h if no thrombolysis, or per post-tPA protocol)

- ☐ Aspirin 160–325 mg PO/PR/NG once daily, first dose within 24–48h of onset (if not thrombolysed) or 24h after tPA once hemorrhage excluded
- ☐ Clopidogrel per dual-antiplatelet protocol if minor stroke/high-risk TIA (per institutional DAPT protocol; typically limited duration)

☐ Therapeutic anticoagulation (e.g., DOAC or warfarin, as clinically indicated) for atrial fibrillation/flutter or another confirmed cardioembolic source. The treating physician will determine the agent, dose, and timing based on infarct size, hemorrhagic transformation risk, and follow-up neuroimaging

8c. Blood pressure management

- ☐ Permissive hypertension if no thrombolysis: treat only if SBP >220 or DBP >120 mmHg (unless comorbid condition requires lower threshold) — physician to individualize
- ☐ If thrombolysed or thrombectomy candidate: maintain BP <180/105 mmHg per protocol
- ☐ Antihypertensive agent/route per protocol: _____
- ☐ Resume home antihypertensives once stable, per physician order — avoid aggressive early lowering

8d. Statin therapy

- ☐ High-intensity statin (e.g., atorvastatin 80 mg PO daily) — start during admission unless contraindicated

8e. Glucose management

- ☐ Fingerstick glucose q6h (or per protocol); treat hyperglycemia to target 140–180 mg/dL
- ☐ Treat hypoglycemia <60 mg/dL per hospital hypoglycemia protocol

8f. VTE prophylaxis

- ☐ Sequential compression devices (start on admission)
- ☐ Pharmacologic prophylaxis (e.g., subcutaneous heparin or LMWH) once hemorrhage excluded and if not already therapeutically anticoagulated — start by hospital day 2

8g. Other supportive medications

- ☐ Antipyretic (e.g., acetaminophen) for temperature >38.0°C — maintain normothermia
- ☐ Bowel regimen (stool softener ± laxative)
- ☐ Home medications reconciled — continue/hold per physician review
- ☐ Glycemic management per institutional protocol; avoid sliding-scale insulin as the sole regimen in patients with diabetes.
- ☐ Avoid medications not indicated for acute ischemic stroke (e.g., nimodipine, unless treating aneurysmal subarachnoid hemorrhage).

9. Consults & Multidisciplinary Care

- ☐ Neurology / Stroke service
- ☐ Physical Therapy
- ☐ Occupational Therapy
- ☐ Speech-Language Pathology (swallow and communication evaluation)
- ☐ Case Management / Social Work — discharge planning, rehab placement, insurance
- ☐ Dietitian, if dysphagia or diabetes management needed
- ☐ Pharmacy — medication reconciliation and anticoagulation dosing
- ☐ Palliative Care, if goals-of-care discussion indicated
- ☐ Neurosurgery, if hemorrhagic conversion or malignant edema risk

10. Patient & Caregiver Education (document in chart)

- ☐ Stroke warning signs (F.A.S.T.) and when to call 997
- ☐ Individual risk factor modification (BP, diabetes, cholesterol, smoking, activity, diet)
- ☐ Purpose, dose, and importance of adherence to discharge medications
- ☐ Follow-up appointment(s) scheduled: Neurology/Stroke clinic, PCP, rehab
- ☐ Written stroke discharge instructions provided

Ordering Physician Authorization

Physician Signature: _____ Date/Time: _____

Print Name: _____ Pager/Contact: _____

References: American Heart Association/American Stroke Association Get With The Guidelines–Stroke program measures; 2019 AHA/ASA Guidelines for Early Management of Acute Ischemic Stroke (and subsequent focused updates); The Joint Commission Primary and Comprehensive Stroke Center certification requirements. This template should be updated to reflect the current guideline version at the time of institutional adoption.